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CMS RFI Section E: Technology Vendors, Data Providers, and Networks

We wish to thank the Centers for Medicare & Medicaid Services for the opportunity to submit public comment on their Health Technology Ecosystem Request for Information. Preventive care allows patients to catch potential health issues early, when they are more treatable. Yet, utilization rates for these services remain stubbornly low. On average, just 40% of patients with commercial healthcare insurance coverage access their free preventive care benefits available from their employer.[1] The rates are far worse for those without coverage.

The consequences of this lack of engagement are severe. Half of all adults have high blood pressure, but only 21% have it under control. Forty-two percent of adults have multiple chronic conditions, and 35% have undiagnosed prediabetes and 40% undiagnosed have high cholesterol.

Reperio Health was founded in 2020 with the mission that every individual should have simple and convenient access to preventive care services. The company provides an innovative at-home kit for patients who wish to understand their biometric data from the convenience of home with instant results.

Patients complete a screening by pairing the Reperio Health mobile app on their smartphone to the kit and collect their health data. Patients without a mobile phone are provisioned one by request. Data collected includes: blood pressure, resting heart rate, body mass index, relative fat mass, total cholesterol, HDL cholesterol, LDL cholesterol, triglycerides, and glucose. Patients are presented with their health data immediately and choose their return method for their kit. All patients have access to 24/7/365 Customer Experience support via chat or email (phone, upon request) for any delivery, return, or screening questions.

Reperio Health's experience serving employers, health plans, hospitals and health systems in their short tenure has yielded remarkable results for their 30+ clients:

- 96% of end users successfully complete the at-home kit
- End users usually take less than 30 minutes to complete their kit
- Customer satisfaction rating is 95%+
- The typical opt-in rate for Reperio Health screenings is about 60% in a given employee population
- Reperio Health can significantly increase participation rates: over 200% for a client compared to the same period of time the previous year
- Most clients realize a 4-8x return on their investment, usually greater

[1] Springbuk internal analysis performed for Reperio Health, 2025.

We look forward to the continued collaboration of CMS with the health technology community to improve access to critical technologies that can empower patients in their wellbeing.

In health,

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TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Reperio Health is an at-home preventive care screening with instant results. We aid in the identification of individuals with health risks before they become high-cost claimants. We strongly believe in encouraging innovation in digital health products for Medicare beneficiaries and caregivers.

To stimulate developer interest in the short term, we suggest CMS creates a sandbox environment for testing Medicare-focused digital health solutions, establishes clear reimbursement pathways for digital preventive care (like Reperio Health's at-home screenings), creates fast-track certification paths for digital health startups, and establishes data-sharing frameworks that support real-time access to beneficiary data.

In the long term, CMS should develop value-based care models that reward early detection and prevention, which aligns with Reperio Health's ability to identify undiagnosed chronic conditions early. Reperio Health's 96% completion rate and instant results demonstrate the engagement potential when barriers are removed. In addition, CMS can support ecosystem growth by subsidizing infrastructure that reduces development overhead and expands access to trusted health information exchanges like QHINs.

TD-2a. What additional data would be most valuable if made available through CMS APIs?

The most valuable data to be made available through CMS APIs are clinical notes, social determinants of health data, diagnostic images, near real-time pharmacy data, real-time biometric data, and care gap information. Reperio Health generates instant biometric data (blood pressure, cholesterol, glucose, BMI and more) that could populate CMS APIs to support population health management. Integration of Reperio Health’s data with claims data would enable predictive analytics for chronic disease management.

TD-2b. What data sources are most valuable alongside the data available through the Blue Button 2.0 API?

Data from QHINs, biometric screening data, wearable device data, and pharmacy adherence data complement Blue Button 2.0. Reperio Health's comprehensive screening data (key health markers) provides clinical-grade data outside traditional healthcare settings, filling gaps where patients don't regularly visit providers. This is critical given the 43-day average wait for primary care appointments.

TD-2c. What obstacles prevent accessing these data sources today?

Siloed systems, inconsistent data standards, and limited real-time access restrict access and innovation contribute to data access problems today.

TD-2d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

CMS and ASTP/ONC should consider including home diagnostic device APIs, virtual care platform APIs, and care coordination APIs to better support patients and providers. Reperio Health's virtual consultation platform integrated with instant diagnostic results exemplifies how APIs can connect home-based care with clinical decision-making. This addresses the 63% of members who prefer telehealth options, and who may otherwise not seek medical care at all.

TD-3a. What are the challenges and benefits of digital identity implementation?

The benefits of digital identity implementation are enhanced security, reduced fraud, and streamlined access to care.

The challenges of digital identity implementation include privacy concerns, implementation costs, and digital literacy barriers.

TD-3b. How would requiring digital identity credentials (e.g., CLEAR, Login.gov, ID.me) impact cybersecurity and data exchange?

Requiring digital identity credentials, such as CLEAR, would create a more secure and standardized identity ecosystem, ensuring patient information is only accessed by authorized users. However, for a Medicare audience these tools may not be as readily adopted.

TD-3c. What impact would mandatory use of the OpenID Connect identity protocol have?

Mandatory use of the OpenID Connect identity would standardize the authentication across healthcare platforms, reducing integration complexity and improving interoperability. For example, Reperio Health's platform would benefit from standardized identity protocols when sharing screening results with provider systems and health information exchanges like QHIN, increasing the likelihood that patients will receive prompt follow up from providers when needed.

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS can better encourage use of open, standards-based, publicly available APIs by tying reimbursement incentives and certification eligibility to FHIR-based, open APIs. CMS can shift the ecosystem away from proprietary systems. Reperio Health is very flexible and can easily leverage APIs to exchange information.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information? Who should publish it and should users bear a cost?

A nationwide provider directory would dramatically improve care coordination and reduce information blocking. CMS should publish it as a public good, funded through existing programs. Reperio Health's virtual clinicians accessing medical history from QHIN demonstrates how connected systems improve clinical decision-making when providers can easily locate and access relevant endpoints.

TD-6a. What unique interoperability functions does TEFCA perform?

TEFCA performs unique interoperability functions - specifically, it provides standardized frameworks for secure, nationwide health information exchange. It enables the type of medical history access Reperio Health's clinicians utilize from QHIN during virtual consultations, creating a fuller picture of the member's health that improves clinical decision-making.

TD-6b. What existing alternatives should be considered?

Alternatives to TEFCA for consideration are direct trust agreements, CommonWell, and Carequality networks.

TD-6c. Are there redundant standards, protocols, or channels that should be consolidated?

Yes, there are redundant standards and consolidating around FHIR R4, USCDI, and TEFCA frameworks would reduce complexity. Multiple proprietary interfaces create barriers to innovation. Reperio Health's streamlined approach to data integration shows how standardization enables better user experiences and clinical outcomes.

TD-7a. To what degree has USCDI improved interoperability and exchange and what are its limitations?

In terms of improving interoperability and exchange, USCDI has provided valuable standardization but is limited in scope for comprehensive care management. It lacks detailed biometric data elements that companies like Reperio Health generate (relative fat mass, detailed lipid panels and more), which are crucial for preventive care and chronic disease management.

TD-7b. Does it contain the full extent of data elements you need?

No, USCDI does not contain the full extent of data elements needed. Specifically, USCDI lacks comprehensive biometric and diagnostic data elements needed for effective population health management. Reperio Health provides these very data points.

TD-7c. If not, is it due to USCDI format limitations or its use?

USCDI's lack of data elements is due to both the format, which lacks flexibility, as well as inconsistent adoption.

TD-7d. Would adding more data elements to USCDI add value or create challenges? How could those be addressed?

Adding more elements like detailed lipid panels, continuous biometric monitoring, and home diagnostic results to USCDI would add significant value for preventive care. Challenges include complexity and implementation burden, which could be addressed through phased implementation and vendor support programs. Reperio Health can provide many of these needed data points, and is very agile in how data is shared.

TD-7e. Would you prefer a non-proprietary but less structured format for better data coverage even if it requires more processing?

Yes, Reperio Health would prefer a non-proprietary, but less structured format, for better data coverage for innovation in preventive care. Flexible formats would enable inclusion of novel diagnostic data like Reperio Health's instant biometric results. The processing burden is manageable with modern analytics tools, and the clinical value of comprehensive data outweighs structural complexity.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

The most effective certification criteria and standards under the ONC Health IT certification program are mandatory FHIR-based APIs, testing across real-world use cases, ability to integrate with national HIEs, API interoperability, patient access capabilities, and clinical decision support integration. Reperio Health's technology is able to accommodate multiple integration types to share data from our at-home preventive care screening.

TD-9a. What are the benefits of prioritizing API-enabled capabilities over software functionality for certification?

The benefits of prioritizing API-enabled capabilities over software functionality for certification are that it enables innovation, reduces vendor lock-in, and improves interoperability. Reperio Health's ability to share results from our at-home preventive care screening via information exchange and integrate with health plan systems demonstrates how API-first approaches enable flexible, scalable solutions that improve care coordination.

TD-9b. What would be the drawbacks?

The drawback to prioritizing API-enabled capabilities over software functionality for certification is the potential confusion without strong governance and education. Also, the complexity could prove burdensome for smaller providers and increase implementation costs. Reperio Health is able to plug into APIs to share crucial biometric data for a population without contributing to complexity.

TD-9c. How could criteria require APIs to support exchanging data from all aspects of a patient's chart?

Criteria to support exchanging data from a patient's chart should include requiring FHIR R4 compliance for all data elements, mandating bulk data export capabilities, and establishing data completeness standards. Also, making access to structured and unstructured data (e.g., PDFs, faxes, scanned files) a baseline standard. Reperio Health can contribute to this effort by accommodating these criteria to share our biometric test results with health systems and health plans.

TD-9d. What CMS policy changes could motivate providers to respond well to API-based data requests?

Ideas for CMS policy changes to motivate providers to respond well to API-based data requests are to tie quality scores and reimbursement to API responsiveness and to establish data sharing incentives in value-based care contracts.

TD-9e. How could EHRs capable of bulk data transfer reduce provider burden for reporting quality data? What capabilities and concerns exist?

EHRs that are capable of bulk data transfer can reduce provider burden for reporting quality data by automating quality reporting and reducing manual data entry. Bulk export tools can streamline submissions and enrich insights. Reperio Health's data, from blood glucose to relative fat mass to total cholesterol, would aid in enriching these insights.

TD-10. What further steps should ONC take to implement API conditions allowing access, exchange, and use of health info without special effort?

In order to allow access, exchange, and use of health information without special effort, ONC should establish clear technical standards, provide implementation support & user-facing documentation, and create enforcement mechanisms for API availability. Reperio Health's technology is able to adjust to updated technical standards for data access, exchange and use.

TD-11a. Should EHI export capability specify standardized API requirements?

Yes, EHI's export capability should specify standardized API requirements because this would improve portability and reduce vendor lock-in. Reperio Health is proud of our adjustability to accommodate requirements as they evolve, so we can share important population health and individualized data.

TD-11b. Are there specific workflow aspects that could be improved in EHI export?

Yes, Reperio Health's suggested workflow improvements in EHI export are automated data validation, member consent, and real time patient-initiated exports need improvement. Reperio Health's users appreciate the fact that our individual results are instant, as anything in real-time allows for immediate action based on the information surfaced.

TD-11c. Should CMS consider policy changes to support EHI export use?

Yes, CMS should consider policy changes to support EHI export use. Specifically, Reperio Health suggests incentivizing patient-controlled data sharing and supporting innovative care models. Reperio Health already supports patient-controlled data sharing of data.

TD-12. Should CMS endorse non-CMS data sources and networks? What criteria should be considered?

Yes, CMS should endorse non-CMS data sources and networks. CMS should prioritize security, member utility, and ability to support value-based care. Reperio Health's reporting supports value based care by surfacing population health insights and segmenting the population for risk stratification.

TD-13a. What new opportunities could APIs accessing full EHI enable?

New opportunities that APIs accessing EHI enable are comprehensive population health management and predictive analytics for chronic disease. Reperio Health offers real-time data on predictors for chronic disease, such as blood glucose.

TD-13b. What are the primary obstacles to accessing full EHI?

The primary obstacles to accessing full EHI are the technical complexity, privacy concerns, and provider resistance. Reperio Health partners with many providers who appreciate the simplicity of the Reperio Health screening process and also the ease of accessing the biometric results we collect.

TD-13c. What are the tradeoffs between USCDI and full EHI with flexible data processing?

The tradeoffs between USCDI and full EHI with flexible data processing are that USCDI provides standardization but limits innovation while full EHI enables comprehensive care but requires more processing. Reperio Health's model suggests the clinical value of comprehensive data (instant health markers) justifies the additional complexity for better patient outcomes.

TD-14a. How many endpoints is your network connected to? What types and regions? Are they searchable by NPI or org ID?

Reperio Health has multiple endpoints our network is connected to. Reperio Health connects to health information exchanges like the QHIN, specifically Kno2, to access medical history during virtual consultations. Our on-demand virtual consultations demonstrate scalable connectivity for nationwide preventive care delivery to both urban and rural populations.

TD-14b. How are connections established (e.g., FHIR, TEFCA, proprietary APIs)?

Reperio Health establishes connections through health information exchanges and direct integration with health plans, and other third parties.. The platform's ability to share results via information exchange suggests use of standardized protocols rather than proprietary approaches.

TD-14c. Do you interconnect with other networks? Under what frameworks?

Yes, Reperio Health interconnects with other networks via national interoperability frameworks and vendor APIs. This allows us the flexibility needed to thrive in partnership with many different types of organizations that aim to improve healthcare.

TD-15a. How would increased use of bulk FHIR improve use cases and data flow?

Increased use of bulk FHIR would enable population health analytics, automated quality reporting, and comprehensive care coordination. Reperio Health's instant generation of multiple health markers demonstrates the type of rich data that bulk FHIR could efficiently distribute for population health management.

TD-15b. What are the potential disadvantages of using bulk FHIR?

The potential disadvantages of using bulk FHIR are the privacy risks from large data transfers and the need for consent orchestration.

TD-16a. What are tradeoffs of point-to-point vs. shared network models?

There are tradeoffs of point-to-point versus shared network models. Point-to-point offers simplicity and control but limits scalability; shared networks enable broader connectivity but increase complexity. Reperio Health can accommodate both scenarios.

TD-16b. Do current rules encourage scalable participation? What changes would improve alignment?

Current rules create barriers to innovative care models because custom integrations are common. Changes to current rules should include recognition of home-based diagnostics, virtual care reimbursement, and incentives for data sharing. Reperio Health supports regulatory efforts to require participation with TEFCA to enable nationwide interoperability efforts.

TD-17. What role should CMS or ONC play in ensuring viable healthcare data-sharing networks with enough supply and demand?

To ensure viable healthcare data-sharing networks with enough supply and demand, CMS should create incentives for participation, establish technical standards, and provide implementation support. The success of companies like Reperio Health in improving health outcomes while reducing costs demonstrates how proper network effects can drive adoption when value is clear.

TD-18a. Can you share examples of practices that may constitute information blocking, including non-responsiveness and failure cases?

Examples of practices that constitute information blocking, including non-responsiveness and failure cases, are the delayed response to data requests, the refusal to recognize third-party apps, excessive fees for data access, and technical barriers to integration constitute information blocking.

TD-18b. What additional policies could discourage information blocking by providers?

Additional policies that could discourage information blocking by providers are financial penalties tied to quality scores, positive incentives for data sharing, patient access requirements, real-time compliance audits, and consequences for repeated blocking.

TD-18c. Are there actors under PHSA 3022(a)(1) that lack information blocking disincentives?

Actors under PHSA 3022(a)(1) that lack information blocking disincentives are provider groups outside of ACOs, tech vendors, and health information networks. Reperio Health is pleased to partner with anyone supporting our end users with their healthcare needs by sending data as appropriate.

TD-19a. What are shortcomings in price transparency content, format, delivery, and timeliness?

Shortcomings in price transparency content, format, delivery and timeliness are complex and non-standard formats, delayed updates, and lack of consumer-friendly presentation limit effectiveness. Reperio Health spent over two years perfecting our consumer-facing app to be inclusive and make the preventive care screening process as user friendly as possible.

TD-19b. Which workflows benefit most from functional price transparency?

The workflows that benefit the most from functional price transparency are preventive care planning, out-of-network care navigation, and high-deductible plan decisions. Reperio Health's value proposition is centered on preventive care, as we identify multiple health markers and offer instant results.

TD-19c. What improvements would be valuable for patients, providers, or payers?

An imperative improvement that would be valuable for patients, providers and payers alike is unified API access to pricing. Reperio Health believes deeply in providing easy access to healthcare information.

TD-19d. What would further motivate development of solutions for price transparency?

To further motivate development of solutions for price transparency, Reperio Health suggests linking transparency to plan ratings, member experience scores, reimbursement tied to transparency compliance, consumer demand for clear pricing, and demonstrated value in improved accessor premium incentives. Reperio Health appreciates these metrics, and consistently receives customer satisfaction scores of greater than 95%.